

إسم المريض رابعياً : مبروكه صبرى السيد صالح

المحافظة : المنوفيه

الحى / المدينة : كفر داود

العنوان : ---

رقم البطاقة : 28404011803562 رقم التليفون : 01062242652

تاريخ تسجيل الإستمارة : 01-10-2026

العمر(تاريخ الميلاد) : 01-04-1984 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع:

التوقيع:

التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج :-

ملاحظات : ---

رئیس

المجلس الطبي المتخصص

مدير عام

مديرية الشؤون الصحية

ختم

شعار الجمهورية

--

Name: مبروكه صبرى السيد صالح National ID: 28404011803562	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٤/٠٨ ٢٨٤٠٤٠١١٨٠٣٥٦٢

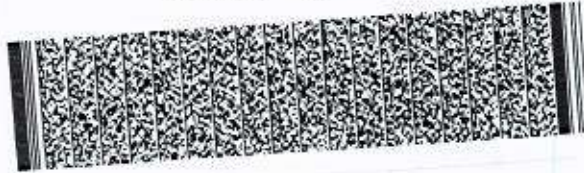
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أنفى مسلمة



البطاقة سارية حتى ٢٠٣١/٠٨/٠٦

١٢



محافظة كفر داود
محافظة كفر داود

مبروكه

صبرى السيد صالح

كفر داود

مركز السادات - المنوفية

١٩٨٤/٠٨

٢٨٤٠٤٠١١٨٠٣٥٦٢

KR5963204



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
صحة
مصر

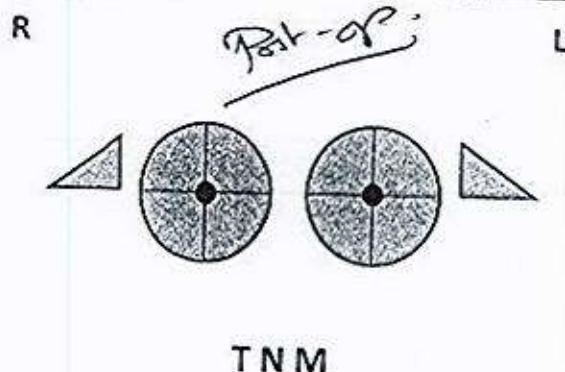
MDT FORM

161 4 / 2025 G

Name	ميردخا مبري سيد صالح	MRN	٢٠٢٢
Age	41 yr	ID	
Menopausal status	Postmenopausal	Phone	01093395770
NRS SCORE		Marital status	Married
BMI		Family history	11-580-429
CO morbidity		REASON OF MDT	Postoperative
Pt Received Neoadjuvant			

Clinical Data

BX: IDC			
ER: 8/8	PR: 8/8	HER 2: 3	Ki67:
Sonomammography:			
Mets Workup: (4) Post MRM, Post Neoadjuvant chemotherapy.			



Recommendations:

Pathology Report: Tx, No, Partial pathological response.

MDT OUTCOME & PLAN

Pt was designated Tx in view of lympho-vascular invasion !!! → review pathology report of reported Non-PCR → ① TDM1 (14 cycles) ② PORT (40 Gy) ③ 5 (AI + 1- Tamoxifen) 5 yr.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: مبروكة صبري السيد		
MR. : 6944		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient is a 65-year-old female with a history of left breast cancer, stage cT4bN3M0, IDC, ER 8/8, PR 8/8, HER2+. The patient underwent left mastectomy with lymph node dissection (LVSI, N0/10). She is currently receiving adjuvant therapy with Kadcyla (trastuzumab emtansine) 260 mg and Femara (letrozole) as part of a hormone-targeted regimen, with Zoladex (goserelin) added for endocrine support. The patient has completed nine cycles of Kadcyla and is scheduled to continue with cycle 10. The patient's treatment plan includes ongoing Kadcyla 260 mg infusions and oral Femara, with supportive laboratory monitoring. The next cycle of Kadcyla is scheduled following acceptance of the pharmacy visit for C 11 Kadcyla 260 and Femara.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
		الطبيب المعالج : dina makram
يعتمد ,		

Patient Name : الأستاذة / مروة صبرى السيد صالح	Request No. : 13008644
Patient No. : 28404011803562	Request Date : 08/09/2024
Path No. : SR-1299/24	Verification Date : 18/09/2024
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide revision.

Gross Examination:

One paraffin block labeled by (201804/24) and three slides were received.

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- DCIS: Positive (comedo).
- Angiolymphatic invasion: Negative.
- Perineural invasion: Negative.
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast cancer, Slide revision:
invasive duct carcinoma, grade II, with DCIS (comedo).

PI:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).

Verified by :
Dr.Toka Medhat

Patient Name	: الأستاذة / مبروكة صبرى السيد صالح	Request No.	: 13008644
Patient No.	: 28404011803562	Request Date	: 08/09/2024
Ref No.	: SR-1299/24	Verification Date	: 18/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr. Toka Medhat

Patient Name : الأستاذ / مبروكه صبرى السيد صالح	Request No. : 13009873
Patient No. : 28404011803562	Request Date : 15/03/2025
Path No. : 1034/25	Verification Date : 07/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 28x20 cm with covering skin ellipse measured 22x15 cm.
- * Tumor extent: A greyish white firm tumor bed site.
- * Size: 16x7 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 2 cm away from deep margin.
- * Attached axilla :Fibrofatty tissue measured 8x7cm entangling 10 L.Ns up to 1.5x1 cm .

Microscopic Examination:

- * Histological type: No residual tumor .
- * Lymphovascular invasion: Minimally positive.
- * Ductal carcinoma in situ: No.
- * Skin involvement: Free nipple .
- * Lymph node involvement: Free lymph nodes (0/10)
- * Resection Margins: Free deep margin.
- * Histologic response to therapy: Pathological partial response.

Diagnosis:

Left breast cancer post NACT, MRM specimen:

No residual tumor remnants.

- Positive minimal lymphovascular invasion.
- Free deep margin.
- Free nipple.
- Free lymph nodes.

Verified by :

Dr.Menan Hossam

Printing Date : 08/04/2025 13:46:44

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

مستكشفة
د. منة هوسام

Patient Name	: الأستاذ / مبروكه صبرى السيد صالح	Request No.	: 13009873
Patient No.	: 28404011803562	Request Date	: 15/03/2025
Path No.	: 1034/25	Verification Date	: 07/04/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Pathological partial response.
- Stage: ypTx, ypN0.

Comment:

There is no invasive tumor or DCIS in breast tissue , yet there was very few positive lymphovascular invasion so staged as (Tx). Lymphvascular invasion is considered bad prognostic factor.

Signature
Dr. Rana Fathy
Dr. Menan Hossam

Verified by :
Dr.Menan Hossam

Printing Date : 08/04/2025 13:46:44

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH

GENDER: Female

DATE : 19-Aug-2024 11:16 AM

PATIENT ID: 20224011103

AGE: 40 y

ORDER: Z109332,Z109333

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening

TECHNIQUE:

- CC and MLO view
- Complementary ultrasound

REPORT:

- ACR:D

Left breast:

- UIQ posterior third indistinct dense lesion with overlying suspicious microcalcification extending anteriorly in linear pattern measuring 6.6X2.2 cm, about 9 cm from the nipple corresponding by US to 11 o'clock irregular hypoechoic lesion measuring 3.3X1.6 cm, about 0.6 cm depth from the skin.
- Multiple (more than 3) axillary level I lymph nodes, measuring 2.7X2 cm.
- no infra or supraclavicular lymph nodes.

Right breast:

- UIQ posterior third focal asymmetry corresponding to normal glandular tissue.
- No spiculated mass lesions or grouped microcalcifications.
- No solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left breast UIQ malignant mass lesion with ipsilateral axillary level I lymphadenopathy (BIRADS 5) for histopathological correlation.
- Right breast UIQ focal asymmetry corresponding to normal glandular tissue (BIRADS 3).

RECOMMENDATION:

Referral to clinic

Reported By:

Dr. Yomna Tarek, MSc

19-Aug-2024 10:43 AM

PATIENT: MABROKAH SABRY ELSAYYED SALEH, 20224011103, Female, 40 y

Report version 1, 19-Aug-2024 10:43 AM

Page 1 of 2



PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH

GENDER: Female

DATE : 01-Sep-2024 09:14 AM

PATIENT ID: 20224011103

AGE: 40 y

ORDER: 3012125,3012126

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS WITH TRIPHASIC ASSESSMENT OF THE LIVER**

1- CHEST:

- Left breast UIQ enhancing mass lesion measuring 22 x 20 mm associated with diffuse parenchymal edema and mild skin thickening.
- Multiple pathologically enlarged left axillary lymph nodes level I & II lymph nodes; the largest is at level I measuring 22 x 23 mm.
- Subcentimetric right axillary level I and II lymph nodes are also noted.
- Few scattered tiny pulmonary nodules; the largest is the right upper lobe posterior segment measuring 3 mm in diameter.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- Mildly enlarged liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicle's dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys (apart from few bilateral tiny calyceal stones; the largest is left middle calyceal measures 3 mm in diameter), adrenals, aorta and I.V.C.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Left adnexal cyst measures 18 x 28 mm
- Minimal pelvic free fluid is noted likely functional.
- No pathological enlarged abdominal or pelvic lymph nodes.

The revised bone window revealed:

PATIENT: MABROKAH SABRY ELSAYYED SALEH, 20224011103, Female, 40 y

Report version 2, 03-Sep-2024 11:17 PM

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1. Dorso-lumbar spondylo-degenerative changes with related sclerosis of VEPs.
2. Right femoral head and left iliac bony tiny sclerotic foci likely representing bone islands.
3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

OPINION:

- Left breast neoplastic process with ipsilateral pathological axillary lymphadenopathy.
- Few tiny pulmonary nodules for follow up.
- No evidence of hepatic metastatic deposits.

Reported By,
Toka Elbehairy, MSc
Dr. Salwa Yahya, MD

03-Sep-2024 11:17 PM

PATIENT: MABROKAH SABRY ELSAYYED SALEH, 20224011103, Female, 40 y

Report version 2, 03-Sep-2024 11:17 PM

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مركز باهيا للتصوير الطبي
بأشعة مقطعية
بأشعة فوق صوتية
بأشعة رنين مغناطيسي



PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH
GENDER: Female
DATE: 19-Aug-2024 11:18 AM

PATIENT ID: 20224011103
AGE: 40 y
ORDER: Z109332,Z109333

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound.

REPORT:

- ACR-D

Left breast:

- UIQ posterior third indistinct dense lesion with overlying suspicious microcalcification extending anteriorly in linear pattern measuring 0.6X2.2 cm, about 9 cm from the nipple corresponding by US to 11 o'clock irregular hypoechoic lesion measuring 3.3X1.6 cm, about 0.6 cm depth from the skin.
- Multiple (more than 3) axillary level I lymph nodes, measuring 2.7X2 cm.
- no infra or supraclavicular lymph nodes.

Right breast:

- UIQ posterior third focal asymmetry corresponding to normal glandular tissue.
- No spiculated mass lesions or grouped microcalcifications.
- No solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

OPINION:

- Left breast UIQ malignant mass lesion with ipsilateral axillary level I lymphadenopathy (BIRADS 5) for histopathological correlation.
- Right breast UIQ focal asymmetry corresponding to normal glandular tissue (BIRADS 3).

RECOMMENDATION:

Referral to Jinc

Reported By,

Dr. Yomna Tarek, MSc 19-Aug-2024 10:43 AM

PATIENT: MABROKAH SABRY ELSAYYED SALEH, 20224011103, Female, 40 y
Report version 1, 19-Aug-2024 10:43 AM



PATIENT NAME: MABROKAH SABRY ELSAYYED SALEH

GENDER: Female

DATE : 09-Sep-2024 12:20 PM

PATIENT ID: 20224011103

AGE: 40 y

ORDER: 3026825

99MTC-MDP BONE SCANClinical Presentation: -

40-year-old female with known left breast cancer, for skeletal survey as part of IS metastatic work-up.

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner.

Findings:

- No hot/cold focal areas of abnormal tracer accumulation that would account for osseous deposits could be detected all-over the surveyed skeleton.
- Mild degenerative changes of the dorsal vertebrae.
- Mild arthritic changes of the knees.
- Warm soft tissue uptake in the region of the left breast: known site of primary malignancy.
- Both kidneys are normally visualized.

Conclusion:

- No scintigraphic evidence of metastatic bone lesions all-over the surveyed skeleton.

Reported By:

Dr. ElShaymaa Hussein, Nuclear Medicine Consultant

10-Sep-2024 01:46 PM

PATIENT: MABROKAH SABRY ELSAYYED SALEH, 20224011103, Female, 40 y

Report Version 1, 10-Sep-2024 01:46 PM

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Ministry of Health, Egypt



Radiology



د. الصحة والسكان
المراكز الطبية المتخصصة
مستشفى اورام فخر السلام

Echo report

3023
MR 24

Name: مروية مبروك
Date: 21/1/2024

EF	63%	55-75%	PWT	1.1	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	3.4	2-3.7cm
LVESD	3.4	2-4cm	LA	3.7	2-4cm
FS	32%	25-45%	RV		0.7-2.3cm(suplne)
SWT	1.09	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA NO SWMA at rest

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

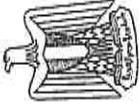
Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion NO

D/Mohamed Ali

Msc cardiology



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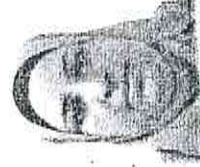
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مسلمة

أنثى

البطاقة سارية حتى ٢٠٣١/٠٨/٠٦

١٢



مبروكه

صبرى السيد صالح

كفر داود

مركز السجلات - المقفلة

٢٠٢٤/٠٨/٠٦

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